VUGA NDAKUMVA

MENTAL HEALTH EDUCATIONAL CONTENT

Chapter 06
Understanding Trauma and Post-Traumatic Stress

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Reader note: This chapter is educational, not a diagnostic test or a substitute for professional care. It does not ask readers to describe or relive traumatic memories. You may skip any activity. If you are currently in danger, prioritize immediate safety and seek local emergency assistance.

This chapter focuses exclusively on mental health responses to distressing or traumatic experiences. It is written for adult readers and people supporting them; guidance for children requires age-appropriate professional adaptation.

Learning outcomes

- Explain trauma, common stress reactions, acute stress disorder and post-traumatic stress disorder (PTSD) in plain language.
- Recognize the core symptom patterns of PTSD without assuming that symptoms confirm a diagnosis.
- Identify practical, low-intensity coping options and the limits of self-help.
- Describe evidence-based treatment and understand when and how to seek professional or emergency support.
- Respond to a person affected by trauma respectfully, without pressuring them to disclose their experiences.

1. Understanding psychological trauma

A potentially traumatic event is an experience involving actual or threatened death, serious injury, sexual violence, or another event perceived as profoundly threatening. Events may be sudden, repeated, witnessed, or encountered in a work setting. Examples include serious road accidents, violent attacks, conflict, disasters, severe medical emergencies and some experiences of abuse. Whether an event is traumatic for a particular person depends on the circumstances and its effects; no one should have to prove that an event was “bad enough” to deserve support.

Psychological trauma describes the emotional and psychological effects that may follow overwhelming or threatening experiences. A person may feel frightened, shaken, numb, angry, ashamed or disconnected. The body’s alarm system can remain highly sensitive even after the immediate danger ends. These responses are not signs of weakness or moral failure. Some people find that reactions settle with time and support; others experience longer-lasting difficulties. [1, 2]

2. Why people respond differently

Two people can live through the same event and have different reactions. Responses are shaped by prior experiences, immediate safety, severity and duration of exposure, injuries, available support, health, age, and whether ongoing threats remain. A supportive environment can help recovery, while continuing danger and social isolation can make recovery harder.

The majority of people who experience potentially traumatic events do not develop PTSD. WHO estimates that approximately 5.6% of people exposed to a potentially traumatic event develop PTSD, although risk differs substantially by type of exposure. This is population information, not a way to predict any individual’s outcome. [1]

3. Common reactions in the first days and weeks

Domain	Examples of reactions
Feelings	Fear, grief, sadness, irritability, shame, anger, shock or emotional numbness.
Thoughts	Replaying parts of an event, confusion, worries about safety, difficulty focusing, self-blame.
Body	Tension, headaches, fatigue, changes in appetite, sleep problems, startling easily.
Behavior	Wanting company or solitude, avoiding reminders, seeking reassurance, withdrawing.

Reactions are not identical for everyone. Some people feel very little at first and notice distress later. Others recover without specialist treatment. Persistent, severe, or worsening symptoms deserve assessment, even if the person appears functional at work or at home. [1, 2]

4. Stress responses versus a diagnosis

A reaction after a frightening event is not automatically a mental health disorder. In the early period, it is often most helpful to focus on practical safety, access to essential needs, supportive contact, sleep where possible, and monitoring whether distress improves. A person can seek help at any time; they do not have to wait for a diagnosis.

In clinical systems, acute stress disorder is a defined condition emerging in the first month after a qualifying event, with a particular pattern of symptoms and functional impact. PTSD involves a different set of diagnostic criteria. A clinician considers symptom timing, event history, distress,

functional impairment, alternative explanations and co-occurring conditions. Self-diagnosis from a checklist is unreliable. [2, 3]

5. What is PTSD?

Post-traumatic stress disorder (PTSD) is a mental health condition that can develop following exposure to one or more traumatic events. Core features include involuntary re-experiencing of the event, avoidance of reminders, and an ongoing sense of threat or heightened arousal. Symptoms must also cause significant distress or interference with ordinary activities. Additional changes in mood, thinking, memory, and relationships may occur. [1, 2]

PTSD can begin soon after an event or become noticeable later. Not everyone with intrusive memories or nightmares has PTSD. Professional assessment matters because symptoms may overlap with depression, anxiety, sleep disorders, brain injury, substance-related problems or physical illness.

6. Re-experiencing symptoms

Re-experiencing means the event returns to awareness in unwanted, distressing ways. This can include intrusive memories, upsetting dreams, or flashbacks. During a flashback a person may briefly feel as though the event is happening again, rather than simply remembering it. Sounds, smells, dates or situations can sometimes act as reminders.

The experience can feel frightening and confusing. Trying to forcefully stop every unwanted memory may not work. When a person feels overwhelmed, gentle orientation to the current environment and professional support can be more useful than repeatedly rehearsing the traumatic event alone.

7. Avoidance and withdrawal

People may avoid places, people, conversations, feelings or activities that remind them of what happened. Avoidance can offer short-term relief, especially when the reminder is intense. However, broad avoidance of safe daily situations may gradually restrict work, travel, relationships and ordinary routines. [1, 2]

Avoidance must not be confused with choosing to stay away from actual danger. It is appropriate to avoid genuinely unsafe people and places. Trauma treatment can help distinguish current threats from reminders of past threats, and can address unhelpful avoidance through a carefully planned process.

8. Hyperarousal and feeling constantly unsafe

Hyperarousal describes a heightened state of alertness. A person may scan surroundings, feel “on edge,” startle at ordinary sounds, struggle to sleep, or become irritable. Concentration can be difficult when the nervous system repeatedly signals danger.

The goal of supportive care is not to tell someone that their fear is irrational. Instead, acknowledge the experience, check present safety, and collaborate on appropriate steps. Physical complaints should be medically assessed when they are severe, unusual or unexplained.

9. Changes in mood, beliefs and connection

Trauma can affect a person’s view of themselves, other people and the world. Some people blame themselves for what happened, feel guilt or shame, lose interest in activities, or struggle to trust others. These beliefs may be shaped by danger, threats or harmful messages received during the event. None of these feelings establishes personal responsibility for an assault or other harm.

Emotional numbness may make affection, enjoyment or connection harder. Supportive relationships and appropriate treatment can help, but a person should not be pressured to become sociable or disclose private details before they are ready.

10. Dissociation and feeling disconnected

Some people experience dissociation: feeling detached from themselves, their emotions or surroundings, or finding parts of an experience difficult to recall. A person may describe feeling unreal, emotionally absent or as if they are watching events from outside themselves. Similar experiences can also have other causes.

If someone becomes disoriented, avoid startling or touching them without permission. Speak calmly, identify yourself, offer simple present-focused choices, and seek clinical help when episodes are persistent, severe or impair functioning. Strong inward-focus or visualization activities may worsen discomfort for some people; a person should be free to stop them.

11. Trauma, sleep and daily functioning

Sleep difficulties can follow trauma, including trouble falling asleep, waking frequently, nightmares, and fatigue. Poor sleep may contribute to concentration problems, irritability and low mood. Gentle routines, an adequately safe sleeping environment, and reducing alcohol can support rest, but recurring nightmares or substantial impairment warrant assessment.

Trauma can also affect study, work, caregiving, physical health and relationships. People may experience a mixture of distress and moments of ordinary functioning. Neither visible productivity nor the absence of tears tells us whether someone needs support.

12. Acute stress and early support after an event

In the hours or days following trauma, a supportive response begins with checking immediate safety, urgent medical needs, shelter, food, access to trusted people, and clear practical information. Offer choices and listen if the person wants to speak. Do not insist on a detailed retelling or suggest that everyone must express their emotions in a particular way.

A single compulsory emotional debriefing session intended to prevent PTSD is not recommended. For people who have clinically important symptoms or acute stress disorder, evidence-based trauma-focused care may be appropriate even within the first month, assessed and delivered by qualified professionals. [2, 4]

13. PTSD, complex PTSD and repeated trauma

Some people experience repeated or prolonged traumatic events and may develop broader difficulties involving emotional regulation, sense of self and relationships, alongside the core PTSD symptoms. ICD-11 describes complex PTSD as a distinct diagnosis. This is not the same as saying that every person who experienced repeated trauma has complex PTSD.

Complex presentations may require flexible, coordinated care that considers current safety, housing, family responsibilities and other mental health needs. Assessment should be individualized, culturally sensitive and never based on an online label alone. [2]

14. Trauma across ages and contexts

Children may express distress through changes in behavior, sleep, separation fears, or repetitive play. Adolescents may show withdrawal, irritability, reduced school participation or risky coping behaviors. Older adults may experience trauma symptoms alongside physical illness or bereavement. These signs can have multiple explanations and should not be interpreted without context.

Readers supporting a child should use age-appropriate care, respect safeguarding duties and seek qualified assistance when distress persists. This adult-oriented chapter is not a trauma-treatment manual for children. [1, 2]

15. Practical coping: orienting to the present

> Optional exercise - “Here and now”: (1) Look around and identify two neutral objects; (2) if comfortable, feel the chair or floor supporting you; (3) say silently, “I am noticing where I am now”; (4) choose a small action, such as drinking water or moving to a quieter place. Keep your eyes open if preferred. Stop if this increases distress or disconnection.

Present-focused orientation can help some people manage temporary overwhelm. It is not an exposure therapy and does not remove the need for medical or psychological care. Not every grounding method suits every person, especially those experiencing dissociation or ongoing danger.

16. Practical coping: steady daily anchors

After trauma, large goals can feel unmanageable. A small daily anchor may include one regular meal, a short walk if safe, a predictable wake time, or checking in with a trusted person. Choose activities that are accessible and realistic, not a rigid schedule that creates more pressure.

Examples: A person recovering from a frightening road incident may begin with a quiet morning routine and help arranging transport. Someone experiencing nightmares may create a simple sleep routine and discuss persistent symptoms with a clinician. Neither person needs to “push through” overwhelming exposure on their own.

17. Practical coping: a supportive contact plan

Think of one or two people who listen respectfully and will not pressure you to disclose. Consider what kind of support you would welcome: a practical errand, accompaniment to a health visit, company, or a simple message. A trusted person is someone you choose, not necessarily a relative or authority figure.

If privacy or personal safety is uncertain, avoid storing sensitive details where someone else can access them. Sharing a trauma history is optional for the activities in this chapter. An emergency situation calls for immediate assistance rather than a routine contact plan.

18. What not to do when coping

- Do not force yourself or someone else to recall every detail of a traumatic event as a self-help exercise.
- Avoid alcohol or other substances as the main way to manage distress; they may worsen sleep and symptoms.
- Do not abruptly stop prescribed medicines without consulting a qualified healthcare professional.
- Do not treat avoidance of an actually dangerous situation as a problem to overcome.
- Avoid online content that presents rapid “trauma cures,” unsupervised exposure, or guaranteed results.

19. Evidence-based psychological treatment

There are effective treatments for PTSD. Trauma-focused cognitive behavioral therapies help people process trauma-related memories and beliefs, reduce disabling avoidance, and rebuild daily functioning. Examples include cognitive processing therapy, cognitive therapy for PTSD, prolonged exposure and narrative exposure therapy. These structured treatments are provided by trained practitioners, with monitoring and an agreed plan. [1, 2]

Eye movement desensitization and reprocessing (EMDR) is another evidence-based option for some people with PTSD. It is a structured clinical therapy, not simply moving the eyes while recalling distressing experiences. Suitability, timing and choice of treatment depend on age, circumstances, clinical needs, preferences and available expertise. [1, 2]

20. Medicines and other supports

A qualified prescriber may recommend medication for some adults with PTSD after discussing benefits, risks, preferences and coexisting conditions. Guidelines differ by population and treatment context; medication is not a universal first-line solution. NICE advises against medicines, including benzodiazepines, for preventing PTSD after trauma. For children, treatment recommendations differ and require specialist assessment. [2]

Care can also address depression, anxiety, sleep problems, substance use difficulties, physical injuries or ongoing safety concerns. Improving living conditions and access to supportive relationships may be important alongside psychological treatment.

21. When to seek professional help

Seek an assessment if symptoms persist, worsen, disrupt work or school, affect relationships, lead to frequent nightmares or flashbacks, or make ordinary activities very difficult. Earlier help is appropriate if symptoms are severe, even within the first month after the event. Trauma-related symptoms are treatable; someone does not need to wait until they can no longer cope.

A healthcare worker can ask about symptoms without demanding a graphic recounting in an initial conversation. Assessment may include sleep, mood, functioning, personal safety, medical conditions and substance use. People can ask about confidentiality, available treatment options, costs and referral pathways.

22. Immediate safety and urgent warning signs

> Urgent assistance: If a person is at immediate risk of suicide or serious self-harm, has life-threatening injuries, is in immediate danger, or is severely confused or unable to stay safe, seek emergency medical or protection assistance now. Use verified local emergency services or the nearest emergency department. This handbook does not supply unverified Rwanda hotline numbers.

If it is safe, stay with someone at imminent risk until appropriate help arrives, and remove immediate hazards only when this can be done without placing anyone in danger. Do not promise secrecy about imminent life-threatening risk. When a person is experiencing ongoing violence or coercion, discuss safety privately with qualified support personnel; avoid actions that could expose them to further harm.

23. Supporting someone who has experienced trauma

Start by listening without judgment. Say that they can share as much or as little as they wish. Ask what would help right now and respect their decisions wherever safely possible. Offer concrete support: accompaniment to a clinic, childcare, transport, or assistance finding information. Follow through on commitments.

Avoid demanding details, comparing their experience to another person’s, questioning why they acted a certain way, or promising that recovery will be quick. Reassurance should not minimize genuine danger. If you are a friend, teacher, caregiver or community volunteer, remain within your role and refer for professional care when needed.

24. Four realistic learning scenarios

Scenario A: Distress after a road accident

A university student experiences a serious road accident. For several weeks, loud traffic sounds trigger fear, the student has nightmares, and transport feels difficult. A supportive response is to assess current safety, provide practical assistance and encourage a healthcare assessment if difficulties persist or impair studies. It is not appropriate to force the student to travel immediately as a “test.”

Scenario B: A health worker after an emergency

A health worker who witnessed a distressing emergency becomes irritable and struggles to sleep. The worker may benefit from a confidential, non-stigmatizing conversation with an appropriate professional, rest where feasible and workplace support. Colleagues should not demand detailed storytelling or assume the worker is unable to perform the job.

Scenario C: Persistent fear after repeated threats

A person who faced repeated threats feels constantly on guard and avoids contact with the person involved. The immediate question is whether threats are ongoing. Maintaining distance may be necessary for safety. Professional care can address trauma symptoms while taking protection, privacy and practical needs into account.

Scenario D: A young person’s changed behavior

After a frightening event, an adolescent withdraws from friends and has difficulty concentrating at school. A trusted adult can respond calmly, ask what support is welcome, and arrange a developmentally appropriate assessment. Pressuring the adolescent to tell the whole class what happened would be inappropriate.

25. Guided reflection: choose, do not disclose

This exercise deliberately does not ask you to describe a traumatic event. Skip any question that feels uncomfortable. You may reflect privately without writing anything down.

- What is one situation in which I currently feel relatively safe?
- What is one neutral activity that helps me reconnect with the present?
- Who, if anyone, respects my boundaries and could offer practical support?
- Is there a daily task I could make easier by asking for help?
- Would it help to discuss symptoms or sleep concerns with a qualified professional?

26. A seven-day gentle support practice

Day	Optional activity
1	Notice current practical needs: rest, food, safety or medical care.
2	Try a brief, eyes-open orientation exercise only if comfortable.
3	Choose one achievable daily anchor, without judging your progress.
4	Identify a trustworthy person or professional contact.
5	Make space for a pleasant or meaningful activity, however small.
6	Review whether symptoms are interfering with your day. Consider assessment.
7	Choose what support you want next; there is no fixed recovery timetable.

This is not a treatment course or a promise that trauma symptoms will resolve in seven days. Any day can be repeated, replaced or skipped. For ongoing severe symptoms, seek qualified care rather than relying on the practice plan alone.

27. Knowledge check and explanations

Question	Suggested answer and explanation
Does every traumatic event lead to PTSD?	No. Many people recover without developing PTSD; responses vary.
Can PTSD include nightmares, avoidance and heightened alertness?	Yes. These are common patterns, though only assessment can establish a diagnosis.
Should people be forced to tell the entire story right away?	No. Respect choice, privacy and readiness; compulsory retelling is not recommended.
Is EMDR a do-it-yourself eye exercise?	No. It is a structured treatment delivered by trained professionals.
Should a person seek help before symptoms become unbearable?	Yes. Earlier assessment can be appropriate, especially when functioning is affected.

28. Important terms

Term	Plain-language meaning
Potentially traumatic event	An event involving extreme threat, violence or serious harm.
Intrusive memory	An unwanted and distressing recollection.
Flashback	A powerful experience in which a past event feels present again.
Avoidance	Staying away from reminders, feelings or situations linked to distress.
Hyperarousal	Being unusually alert or reactive to perceived danger.
Dissociation	A feeling of disconnection from oneself, memory or surroundings.
PTSD	A clinically defined condition that can develop after trauma exposure.
Trauma-focused CBT	A structured psychological treatment addressing trauma symptoms.
EMDR	A structured therapy used by trained clinicians for some trauma presentations.

29. Key messages to remember

- Trauma reactions are real and vary widely; having them does not indicate weakness.
- Most people exposed to a potentially traumatic event do not develop PTSD.
- PTSD can be assessed and treated; recovery is possible.
- Safety and voluntary choice come before exercises or detailed storytelling.
- Grounding and routines are optional coping supports, not substitutes for trauma-focused therapy.
- Seek professional assessment for severe, persistent or disabling symptoms.

30. Source notes and references

[1] World Health Organization. (11 September 2026). Post-traumatic stress disorder. https://www.who.int/news-room/fact-sheets/detail/post-traumatic-stress-disorder

[2] National Institute for Health and Care Excellence (NICE). (2018, current online guideline). Post-traumatic stress disorder, NG116: recommendations. https://www.nice.org.uk/guidance/ng116/chapter/recommendations

[3] American Psychiatric Association. Diagnostic and Statistical Manual of Mental Disorders, Fifth Edition, Text Revision (DSM-5-TR). Referenced for clinical distinctions; formal criteria are not reproduced.

[4] World Health Organization, War Trauma Foundation and World Vision International. (2011). Psychological first aid: Guide for field workers. https://www.who.int/publications/i/item/9789241548205

31. Publication and safeguarding review

Status: Evidence-informed editorial draft. Not yet approved for clinical publication. Before release through VUGA NDAKUMVA, a licensed mental health professional should verify clinical wording, a safeguarding specialist should review emergency pathways, Rwanda-based partners should verify local support services, and expert translators should validate Kinyarwanda and French adaptations. Accessibility and user-comprehension checks are also required.

Editorial boundary: This material describes trauma and PTSD but never diagnoses an individual, provides unsupervised trauma processing, or supplies unverified emergency numbers. Case examples are fictional teaching illustrations.